جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : سميحة عبد المعبود عباس سيد
المحافظة : القليوبيه **الحي / المدينة :** القناطر الخيريه **العنوان :** القناطر الخيرية
رقم البطاقة : 28501261402925 **رقم التليفون :** 01114647089 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 26-01-1985 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سميحة عبد المعبود عباس سيد
National ID: 28501261402925

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

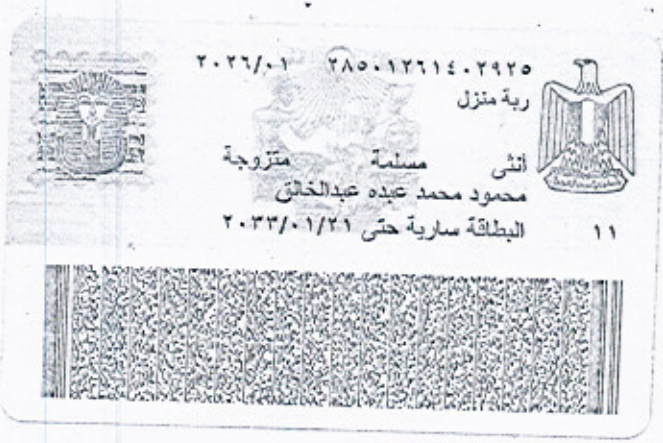
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Supportive and palliative Treatment**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

وزارة الصحة والسكان
إدارة رئيس الجمهورية لدعم صحة المرأة المصرية

MDT FORM 12 / 9 / 2022 G

Name	[illegible]	MRN	[illegible]
Age	38	ID	[illegible]
Menopausal status	Pre	Phone	8308849
NRS SCORE	2	marital status	Married
BMI	over weight	Family history	-ve
Co morbid'ty	None	REASON OF MDT	

Clinical Data

Bx: IDC GII

ER:	PR:	HER 2:	KI67:
6\8	4\8	+3	25%

Sonomammography: S\P left breast surgery showing UOQ path altered parenchym & ipsilat path axil LNs

Mets Workup: CT/MP Free

Bone scan

R: Post. Lpchg

T3 N1 Mo

N.B. original path show +ve margins

Recommendations:

- Pt underwent Lpchg on 6/8/2022
- Awaiting re margins
- Post-op re staging ⟷ Surgical oncology ⟷ pathology ⟷ Radiology (on)

MDT OUTCOME & PLAN

Start NACT (TCHP x6 C-CV support). Made (regular assessment of response the [illegible] (on finish [illegible])

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	psychiatric
SIGNATURE					
12/10/22					

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: سميحه عبد المعبود عباس	
MR. : 6475	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 41-year-old individual with hormone-receptor-positive breast cancer (ER 6/8, PR 4/8), HER2-positive (3+), and Ki-67 25%, staged clinically as T3 N1. The initial management consisted of neoadjuvant chemotherapy combined with anti-HER2 therapy, followed by a left modified radical mastectomy performed in August 2022. Post-operative pathology confirmed the need for postoperative radiotherapy (PORT). The patient is currently receiving adjuvant endocrine therapy with Zoladex and Femara, and adjuvant anti-HER2 therapy with Herceptin, which was initiated today for a planned duration of three months.

تحريرا في ,

التاريخ :	21-09-2026	**الوقت :**	

الطبيب المعالج : علياء احمد أبو النصر شهاب الدين

يعتمد ,

Patient Name	: الأستاذه / سميحه عبدالمعبود عباس مسابر ابو زيد	Request No.	: 13007593
Patient No.	: 28501261402925	Request Date	: 24/08/2022
Path No.	: SR- 619/22	Verification Date	: 12/09/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 37 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical data
Left breast mass

Nature of specimen:
Slide revision

Gross examination:
- 1 block (29968/22) + 6 slides.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR,HER2. ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
* Histological type: Invasive duct carcinoma .
* Grade (differentiation +nuclear pleomorphism+mitosis): (3+2+1) Grade II.
* DCIS: Positive
* IPT:
- ER: Moderate nuclear reaction in 50% of tumor cells (Allred score 6/8)
- PR: Weak nuclear reaction in 20% of tumor cells (Allred score 4/8)
- HER2: Strong complete membrane reaction in >10% of tumor cells (score 3+)
- Ki 67: 25% nuclear reaction of tumor cells
* Positive control showed appropriate reactivity.

Diagnosis:
Left breast mass, slide revision :
- Invasive duct carcinoma, Grade II .
- IPT:
ER: Positive (score 6/8).

قسم الباثولوجي

Verified by :
DR.Menna Nabil

Immunohistochemistry
[[DOCTYPE:IHC]]

Patient Name	الاستاذه / سميحه عبدالمعبود عباس صابر ابو زيد	Request No.	13007593
Patient No.	28501261402925	Request Date	24/08/2022
Path No.	SR- 619/22	Verification Date	12/09/2022
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	37 Y	Department	Out Patient
Referreing Doc.		Registered by	Ali mostafa

IMMUNOHISTOCHEMISTRY REPORT

PR: Positive (score 4/8).
HER2: Positive (score 3+).
Ki67: Positive in 25% of tumor cells.

Prof.Amal Hareedy
Dr.Esraa Magdy

Verified by :
DR.Menna Nabil

Immunohistochemistry
[[DOCTYPE:IHC]]

PATHOLOGY REPORT

Code:	29723517681 -(P 17270-23)			Patient Name:	سميحه عبدالمعبود عباس
Age:	38 Y	Sex:	Female	Specimen Received Date:	6/21/2023
Branch:	Kanater Khairia			Date Report Issued:	04/07/2023
Referred By Prof:	-			Unit Manager:	Prof. Dr. Ahmed Abdelaziz

CLINICAL DATA: A case of increased endometrial thickness with history of breast cancer and hormonal therapy.
SPECIMEN: Endometrial biopsy

GROSS PICTURE: Curetting, totally embedded.

MICROSCOPIC PICTURE: Serial sections examined revealed portions of endometrial tissue fragments, with areas showing uneven glands/stroma ratio. The glands were irregular in shape with cystic forms. They were lined by single layer of cuboidal cells, with irregular secretory changes. The stroma was expanded, showing areas of edema and variable pseudo-decidualization. Tiny portions of benign endometrial polyp with thick-walled vessels and fibrotic stroma were seen. No evidence of atypia or malignancy.

DIAGNOSIS: Endometrial Biopsy:

- **Disordered Proliferative Endometrium, with Hormone-Induced Changes & *Benign Polyp Formation*.**
- **No Atypia or Malignancy.**

Dr. Maha Emad El-deen, M.D.
Lecturer Of Pathology
Faculty of Medicine, Cairo University

NOTE: This report was generated by computer, any manual correction will make it null and void

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Samiha Abdel Ma'boud Abbas Sayed Saber Abou Zaid	Date : 17/08/2022
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 1669	Pat. ID: 17084

Combined bilateral breast mammography and complementary ultrasound

Indication:

History of left breast MDE with pathologically proved IDC.

Technique

- Bilateral MLO and CC views, Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Left breast shows UOQ asymmetry with underlying suspicious clustered of microcalcifications.
- Left diffuse skin thickening.
- Complementary ultrasound revealed left breast ill-defined hypoechoic pathological altered parenchyma with intervening prominent ducts with secretions, roughly measuring about 50 x16mm.
- Another small ill defined hypoechoic solid mass lesion seen at 2 o'clock measuring about 10.5 x6.5mm.
- left diffuse skin thickening.
- Multiple ill defined anechoic cystic collection seen at 12 o'clock and 6 o'clock measuring about 60 x176mm and 65 x29mm respectively.
- No right solid or cystic lesion.
- Normal skin thickness, and nipple-areola complex of the right breast.
- Nonspecific right axillary LN.
- Multiple prominent axillary lymph nodes yet one Pathological left seen with muffled hila, measuring 23.5 x15mm at level I.
- Free supra and infra clavicular ln.

Opinion:

- Left breast pathologically proved carcinoma with UOQ pathological altered parenchyma with ipsilateral one pathological axillary lymph node at level I (BIRADS 6)
- Benign findings right breast (BIRADS 2)

Yours truly

Signature Dr. Nehal Hamdy

17/08/2022 12:55 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

PDF.js viewer

مستشفى العربى
ELARABY HOSPITAL

GAHAR

Patient ID: 0000137235	Patient Name: SAMEHA ABDELMABOOD ABAS SAYED
Procedure:	Study Date: 5/16/2024 1:38:56 PM
Age: 39 Years	Sex: F
Report Date: 5/16/2024 11:54:52 PM	Ref. Phy.: Metwally^MOHAMED^El^Sayed^Awad

Examination: OLANZAPINE 10 MG TAB

Technique: AP and Lateral views

Comparison: No available studies for comparison

Findings:

• Distal tibial shaft eccentric oval shaped sclerotic lesion at its anterolateral aspect causing
contour bulge with narrow zone of transition measures 3.5 X 2 cm yet no periosteal reaction or
soft tissue shadow: picture suggestive of enchondroma
chondroblastoma. for further MR
evaluation.

• No soft tissue abnormality could be detected
• Smooth intact articular surfaces with preserved joint spaces. No loose bodies
• No fracture or dislocation is present
• Normal bone density & texture
• Plantar calcaneal spur noted

Impression:

• Distal tibial shaft eccentric oval shaped sclerotic lesion as described; picture suggestive of
enchondroma or chondroblastoma. for further MR evaluation.
• Plantar calcaneal spur.

Dr. Said Khamis. MSc

عيادات العربى التخصصية الحديثة
مستشفى العربى
Ultrasound
[[DOCTYPE:ULTRASOUND]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
امانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Samiha Abd Al Maaboud Abbas Sayed	Date : 02/02/2023
Exam : C.T CHEST	Age : 38 Year(s)
Medical ID:	Pat. ID: 17084

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Findings:

Known case of left breast cancer shows:

- Few bilateral tiny pulmonary micro-nodules, the largest is seen at the apical segment of right lower lobe measuring about 5 mm. Otherwise, no sizable pulmonary masses, areas of consolidation or cavitary lesions.
- No evidence of pleural effusion or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal non-enhanced appearance of the thoracic aorta and the great vessels.
- The scanned dorsal skeleton shows no sizable lytic or sclerotic lesions.
- Upper abdominal cuts are unremarkable.
- Right sided porta cath is noted.

OPINION:

- ***No CT evidence of active pneumonia.***

Dr. Amr Farouk, M.D.

CT Scan
[[DOCTYPE:CT_SCAN]]

Visit Number:	61122523114	Client Name:	Normal Patient
Patient Name:	سميحه عبدالمعبود عباس	Referred by Prof:	-
Age / Sex:	38 year / Female	Exam Date:	03 Oct 2022
Patient ID:	128766558	Printed:	10/4/2022 7:41:01 PM

NM

Tc-99M MDP SKELETAL SCINTIGRAPHY

HISTORY

38-year-old lady presented with history of left breast carcinoma and referred for metastatic workup.

TECHNIQUE

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer (20mCi Tc99m MDP).

FINDINGS

- **No evidences of any active or cold osseous tracer uptake abnormality that would account for bone metastases.**
- Osteoarthritic changes of both knees are noted.
- Active tooth paste is noted at upper left mandibular ramus for clinical correlation.

IMPRESSION

- ***No metastatic osseous deposits could be detected.***

Dr. Yasser Mohamed, MD
Prof of NM Kasr Elaini
Fellow to Imperial College U.K

Scan Result

Bone Scan
[[DOCTYPE:BONE_SCAN]]

Patient Name: Samiha Abd Al Maaboud Abbas Sayed	Date : 06/07/2025
Exam : Item 1	Age : 40 Year(s)
Medical ID:	Pat. ID: 17084

NON-CONTRAST CT SCAN OF THE CHEST

Findings:



- Early interstitial pulmonary changes are seen involving both lungs manifested by relatively increased lung attenuation mounting to ground glass haziness associated with interstitial coarsening & fine reticular infiltrations with septal thickening & subpleural lines .
- No significant pleural effusion, thickening or masses could be appreciated.
- The cardiac size is not enlarged with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Unremarkable non-enhanced appearance of the thoracic aorta and the great vessels.
- Upper abdominal cuts are unremarkable.

Opinion:

- ***CT findings are impressive of early interstitial lung changes as forementioned for clinico-laboratory correlation.***

Kindly accept my best wishes
Dr. Reham khedr ,,,MD.
Radiology consultant

Signature Dr:Mohamed Hassan
06/07/2025 6:44 PM

Bone Scan
[[DOCTYPE:BONE_SCAN]]